

The National Focal Point survey as evidence for the RespiCast review

A reading of the 2024 NFP survey against the review's least-cost test — what it does and does not license.

Provenance and scope (read first). *Instrument:* the 2024 ECDC survey of National Focal Points (NFPs) for respiratory viruses, **designed to evaluate RespiCompass** and read across here to RespiCast — which bounds what can be asked of it. *Vintage:* fielded in **2024**, when RespiCast was ~1 year old; it informs a late-2026 decision, so its engagement figures are ~2 years old and cannot be read as current use. *Population:* **19 of ~30 NFPs** replied — national surveillance / public-health **consumers**, giving one collective **institutional position** per country (the respondent need never have opened a forecast). They are **not** the contributor modelling network (~9 teams / 28 models) and **not** the website-user survey (results due November); those three must not be conflated. *Non-response:* 19 of ~30 is not random — the absent ~11 plausibly skew **less** engaged, so the engagement figures here are, if anything, **optimistic**; every share also moves ~5 points per country and subgroup counts below are single digits. *The governing question:* the review asks not only what value RespiCast *has* and *could have*, but whether that value could be had **more cheaply**. A benefit counts toward continuation only where the survey gives reason to think a cheaper route would not deliver it.

1. What the survey licenses — and the gap where the cost sits. The strongest finding is a **need: 13 of 19 institutes (68%) have no in-house mathematical modeller**, and only one has a team above five (Figure 1). That is clear evidence ECDC should provide **some** central forecast. It is **silent** on the question that carries the cost — whether that forecast must be a ~28-model community ensemble on a hubverse platform rather than a single centrally-run model. The survey cannot compare the two, and that silence is itself decision-relevant. It sharpens against the stated use: the activity NFPs most expect modelling to inform is **situational awareness** (surveillance, **58%** “likely”), while the demanding forecasting use case — **healthcare-capacity planning** — is the one they expect **least** (**16%** “likely”; 42% “unlikely”). An undemanding use case is one a much simpler product might serve adequately. What a multi-model ensemble buys over a simple central forecast, for situational awareness, the survey cannot say — and should not be read to.

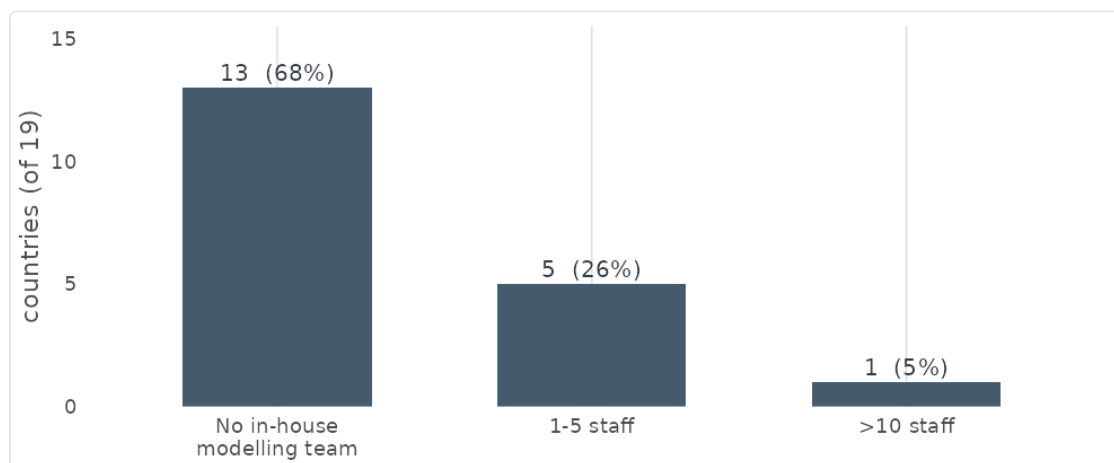


Figure 1. In-house mathematical-modelling staff at the national institute (2024 NFP survey, 19 countries).

Source: code/05_figures/fig_respicast_decision.R.

2. Does the need convert into demand? Read straight. Engagement with RespiCast averages **2.1 / 5** (18 countries), but the average hides the structure that matters. Separating awareness from use: countries with a team engage at **2.2** (n = 5); the no-modeller countries average **2.1** (n = 13) — but that includes **three that were unaware** (score 0), and the **ten aware no-modeller countries engage at 2.7** (n = 10), the highest of any group and level with the mature-forecast ceiling (§3). Two readings were possible in principle — that value is merely *latent*, or the more troubling one that the absence of capacity is **not converting into demand**. The review’s discriminator — do the RespiCast-unaware cluster among the no-modeller countries? — resolves it, at small n, toward the first: **all three “not aware” responses come from no-modeller countries; every country with any in-house capacity was aware.** So the low average is largely an **awareness / distribution gap** among the highest-need countries — cheap to close, favouring *continuation-evolved*; where the product is known, the need **does** appear to convert into demand. This rests on three unaware and ten aware responses: indicative, not robust.

3. But the ceiling is modest. Those aware no-modeller users sit at ~2.7 — which is also where the **COVID-19 Forecast Hub, three years in service, sits** (guidance documents reach 3.9). Forecasts as a class engage **below** guidance; the mature-forecast ceiling looks ≈ 2.7 . So closing the awareness gap would lift the average toward ~2.7, **not toward high use** — even a fully-reached rollout of this product class plateaus at moderate engagement. With the 2024 vintage, 2.1 is an early and probably optimistic reading, and ~2.7 is the realistic ceiling.

4. The realised-value ceiling: a route into decisions. Independent of the product, only **9 of 19 institutes (48%)** report a clear mechanism to integrate modelling into public-health decisions; **6 (32%)** disagree (Figure 2). This caps what any forecast can achieve nationally. Whether ECDC helping to close it is cheap or a cost carried elsewhere is a cost question (§6), not free upside.

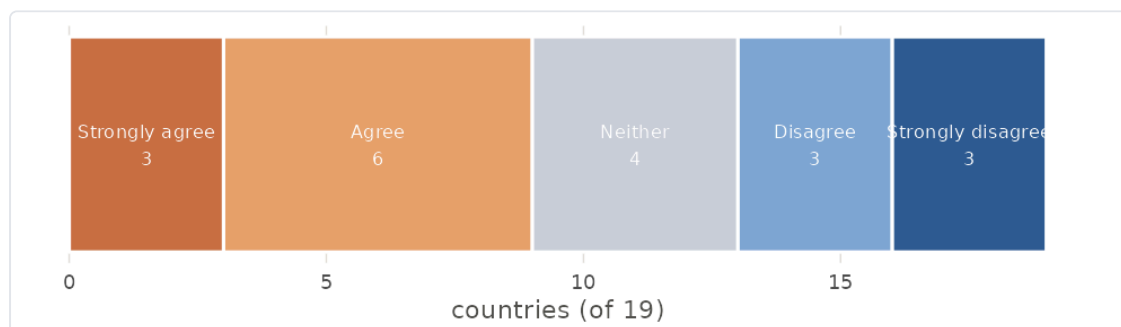


Figure 2. “There is a clear, consistent mechanism to integrate modelling outputs into public-health decisions at my institute” (2024 NFP survey, 19 countries). Source: `code/05_figures/fig_respicast_decision.R`.

5. The beneficiaries’ own ask is a rival for the budget. The clearest single open comment is not a request to improve RespiCast but to fund national capacity: “*An ECDC recommendation for MSs to have adequately resourced and funded national-level modelling capacity ... A sustainable European funding mechanism ... Supranational modelling is useful, but national-level capacity is required to respond to the needs of each MS.*” In least-cost terms this is a **challenger to the hub for the same money**, and it is the beneficiaries’ stated preference. If 68%-no-modeller is the strongest survey argument for a central forecast, this is the strongest survey argument that the same budget might buy more capability by **funding national capacity**. Both come from this survey; both belong before the decision.

6. The improvement levers are cost lines, not free upside. The changes the survey points to are “continuation-evolved” levers, each with an owner and non-trivial effort — costs the least-cost test must weigh, not benefits:

Lever the survey points to	Owner	Order-of-magnitude effort / where costed
Plain-language, interpretable outputs for non-modellers	ECDC (hub / comms)	Low–med; recurring editorial
Short training for NFP consumers	ECDC + NFPs	Med; recurring
Published skill / accuracy track-record	ECDC + contributor teams	Med; needs an evaluation pipeline — not a survey deliverable
A mechanism to use forecasts in national decisions	Member states	Med–high; largely a national process cost
National modelling capacity + funding	Member states / EU	High; budget-scale — a strategic alternative (§5), costed elsewhere

7. What this evidence does not speak to. Internal ECDC value (the ERVISS-integration interviews); surge / emergency-response capacity; forecast **accuracy or skill** (this is stated need and perception, not performance); the **contributor community** (~9 teams / 28 models — a different population); the hub-versus-simple-forecast cost question (§1); and anything **after 2024**.

8. Mapping onto the review’s options (considerations, not a recommendation).

- **Continuation, as-is** — supported only for *a* central forecast, not specifically for the multi-model hub; engagement modest and possibly near a ~2.7 class ceiling.
- **Continuation, evolved** — the cheapest well-evidenced lever is closing the awareness/distribution gap among no-modeller countries; interpretability and a decision pathway follow, at cost (§6).
- **Descoping (core / modularise / caretaker / transfer)** — consistent with the undemanding stated use case (situational awareness; capacity-planning only 16%); a lighter product might serve it. The survey neither rules this in nor out — it does not compare the hub against a simpler forecast.
- **Termination** — not supported: 68%-no-modeller is a clear need for *some* central forecast; but “some forecast” is not “this hub”.
- **Rival use of funds** — the beneficiaries’ own funding ask (§5) is a live alternative for the same budget.

All figures and numbers are generated from the de-identified survey by code/05_figures/fig_respicast_decision.R. Subgroup counts are single digits; percentages are indicative (± 5 points per country) and, given non-response, likely optimistic. A companion comparison — that more NFPs expect both ECDC products to be useful than either alone, with open comments favouring forecasts as more reliable and actionable than long-horizon scenarios — is drawn from a RespiCompass-designed instrument and is not relied on here.